BASELINE → TREATMENT → FOLLOW-UP

Your treatment, tests and checks

Required regimen tests, symptom-triggered investigations and cancer-response tests serve different purposes. Ask who books each test and who explains the result.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks	Full blood count, kidney and liver tests and magnesium; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy-test results. An electrocardiogram (ECG) is indicated with compromised heart function; hearing tests and creatinine clearance are arranged when indicated.	Full blood count, kidney and liver tests and magnesium before each cycle. Check skin, eyes, bowel symptoms and infusion reactions. Ask about hearing, tinnitus, tingling and hydration at each cycle. Report symptoms before every weekly cetuximab visit, even when chemotherapy is not scheduled.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	After the final cetuximab dose, electrolytes including magnesium and calcium are checked monthly for 2 months. Continue review of persistent skin, kidney, hearing, nerve and nutritional problems. Disease examinations and scans are individually planned.
Treatment and appointments	Confirm the diagnosis, aim, prescription and actual calendar.	Cetuximab starts with a 400 mg/m² loading dose over 120 minutes, then 250 mg/m² over 60 minutes weekly. Chemotherapy starts at least 1 hour after cetuximab finishes. Cisplatin 100 mg/m² is given over 2 hours on Day 1 with prescribed fluids before and after. Fluorouracil is 1,000 mg/m² over 22 hours daily on Days 1–4, or the alternative 2,000 mg/m² over 46–48 hours repeated on Days 3–4. Staff confirm the chosen pump schedule. Plan for monitoring during and for at least 1 hour after each cetuximab infusion under the current medicine information; confirm the observation plan with your unit.	Follow the written home plan and keep pump or line appointments where applicable.	Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (National Cancer Control Programme (NCCP) 00207). This is not an automatic continuation.	Record the last dose and follow-up appointments.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Mouth, weight and swallowing	Review oral health, weight, nutrition and safe swallowing.	Discuss mouth pain, intake, weight and hydration.	Report poor drinking, choking or weight loss promptly.	Dietitian and swallowing plans change with symptoms and recovery.	Continue dental, nutritional and swallowing support when needed.
Cancer assessment	Agree staging, pathology and any relevant disease tests.	Discuss symptoms and the planned response review.	Scans or examinations may be added if symptoms require them.	The team sets imaging and cancer-assessment timing.	No universal imaging interval or routine tumour-marker panel is imposed by this regimen.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00417 v7: official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Full blood count

WHEN

At baseline and before each cycle

Checks infection-fighting cells, oxygen-carrying cells and platelets.

Kidney and liver tests

WHEN

At baseline and before each cycle

Guide safe treatment and identify organ problems.

DPD assessment

WHEN

Before the first fluorouracil treatment unless already tested

Checks for reduced ability to break down fluorouracil; results affect treatment choice or dose. Testing cannot exclude every severe reaction. Kidney impairment can affect one blood-based test, so the team chooses the appropriate method.

Hearing assessment

If part of your plan

WHEN

At baseline when clinically indicated; review new hearing or balance symptoms during treatment

Investigates hearing loss or tinnitus and helps judge platinum-related risk.

Electrocardiogram (ECG)

If part of your plan

WHEN

Before treatment if heart function is compromised; additional assessment if symptoms warrant

Records the heart's electrical activity. New chest symptoms require assessment even after a reassuring earlier test.

Magnesium and other electrolytes

WHEN

Magnesium at baseline and before each cycle; magnesium, calcium and electrolytes monthly for 2 months after final cetuximab

Detects mineral changes caused by cetuximab; low levels may cause cramps, weakness or heart-rhythm problems.

Imaging and tumour-specific tests

If part of your plan

WHEN

As individually arranged for the disease and symptoms; no universal marker panel or imaging interval

Assesses the cancer rather than whether a treatment-day blood count is safe.

THE WHOLE TREATMENT JOURNEY

Cetuximab + cisplatin + fluorouracil · 21-day cycle: treatment and checks

Locally advanced or metastatic head and neck squamous cell carcinoma.

Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests and magnesium; DPD testing before fluorouracil unless already tested. Review previous antibody reactions, red-meat allergy, tick bites and any cetuximab allergy-test results. An ECG is indicated with compromised heart function; hearing tests and creatinine clearance are arranged when indicated.	Full blood count, kidney and liver tests and magnesium before each cycle. Check skin, eyes, bowel symptoms and infusion reactions. Ask about hearing, tinnitus, tingling and hydration at each cycle. Report symptoms before every weekly cetuximab visit, even when chemotherapy is not scheduled.	Report new symptoms promptly; extra tests may be needed.	A held or changed dose needs a new clinical review and relevant repeat results.	After the final cetuximab dose, electrolytes including magnesium and calcium are checked monthly for 2 months. Continue review of persistent skin, kidney, hearing, nerve and nutritional problems. Disease examinations and scans are individually planned.

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What each check is for

1 Combination treatment

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

2 Days 8 and 15 within the same combination cycle

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

3

Possible cetuximab-only maintenance, if separately agreed

Follow your agreed treatment calendar

Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Ask / act: Confirm active medicines, written support plan and review criteria.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.47